



SAMPIGE

ONCO DIAGNOSTICS

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Name	Mr Chandrashekar	PET No	PET 15349/25
Age/Sex	73 yrs/M	Pt ID	
Ref/Dept	Dr Nithin Yashas	Date	7.4.2025

- Physiological tracer distribution is seen in the brain, myocardium, kidneys and urinary bladder.

IMPRESSION:

K/C/o Colon carcinoma s/p right hemicolectomy and chemotherapy (compared to previous PET CT done on 18.11.2024):

- No hypermetabolic lesions in anastomotic site or bowel loops
- Interval INCREASE in metabolic activity of several hypermetabolic lesions in liver s/o metastases.
- Interval INCREASE in size and FDG uptake in the portocaval lymph node s/o metastases
- INTERVAL APPEARANCE of hypermetabolic portocaval lymph node s/o metastases
- Interval appearance of non FDG avid subcentimeter nodules in the lungs - concern for metastases - Recommended follow up imaging.
- No significant hypermetabolic lesions anywhere else in the body.
- Scan findings are s/o partial metabolic response to therapy.

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Consultant

Nuclear Medicine Physician



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- **No free fluid in abdomen and pelvis.**
- Esophagus and stomach appear unremarkable
- Gall bladder is shows calculi
- Spleen appears normal in size with no hypoattenuating lesion / focal abnormal FDG uptake.
- Pancreas appears unremarkable. No dilatation is seen in the main pancreatic duct.
- Bilateral kidneys appear unremarkable. No focal nodularity or focal abnormal FDG uptake. No evidence of renal calculi on either side.
- Non FDG avid ill defined nodules noted in the adrenal glands. No FDG avid lesions noted in adrenal glands
- Prostate is unremarkable;
- **Interval appearance of non FDG avid subcentimeter nodules in both lungs.**
Tiny calcific nodule noted in right lung upper lobe. No significant abnormality is otherwise detected in the bilateral lung parenchyma. No evidence of focal abnormal FDG uptake. No evidence of pleural effusion or thickening on either side
- No FDG avid mediastinal lymph nodes
- Note is made of chemoport in right anterior chest wall with its tip in SVC
- Thyroid gland appears unremarkable
- No FDG avid cervical or supraclavicular lymph nodes.
- Mild diffuse FDG uptake in bone marrow is likely reactive. Mild degenerative changes are noted in the spine. Visualized bones otherwise appear normal with no focal lytic / sclerotic lesions or focal abnormal FDG uptake
- Note is made of small vessel ischemic changes in brain parenchyma.



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PET-CT Report

Clinical Indication: K/C/o Ca Colon s/p surgery and chemotherapy.

Technique: Whole body images (base of skull to mid thigh) were acquired in 3-D mode 105 min after i.v. injection of 7.5 mCi of ^{18}F -FDG using a Discovery IQ PET-CT scanner. Water was used as negative oral contrast. IV contrast was also used.. Reconstruction of the acquired data was performed so as to obtain fused PET-CT images in transaxial, coronal and sagittal views.

Comparison: Present scan is compared to previous PET CT done on 18.11.2024

Findings:

- Post surgical changes of right hemicolectomy. No FDG avid lesions noted in anastomotic site
- Bowel loops otherwise appear unremarkable
- Status post liver metastasectomy; Liver appears normal in size. FDG avid multiple hypodense lesions noted in both lobes of the liver - Interval increase in FDG uptake of several lesions as compared to previous scan (reference lesion along the segment V abutting gall bladder measures 1.7 x 1.6 cms; SUVmax 10.4 compared to previous size 1.4 x 1.2 cms; SUVmax 6.1; Reference lesion in segment VIII shows SUVmax 10.4 compared to previous SUVmax 7.1)
- Interval increase in size and FDG uptake in the portocaval lymph node (Size 1.4 x 2.2 cms; SUVmax 6.7 compared to previous size 1.0 x 1.6 cms; SUVmax 3)
- Interval appearance of FDG uptake in periportal lymph node (Size 1.8 x 2.4 cms; SUVmax 6.4)
- No other FDG avid abdominal or pelvic lymph nodes